

1. Administrative & Study Identification

Official Study Title: A Phase 4 Study of Nivolumab in Combination With Ipilimumab in Patients With Previously Untreated Advanced Renal Cell Carcinoma and Intermediate-or Poor-risk Factors Conducted in India **Study Title:** A Study to Evaluate the Safety and Efficacy of Nivolumab With Ipilimumab in Participants With Untreated Advanced Kidney Cancer Conducted in India **Protocol Number:** CA209-7C9 **NCT Number:** NCT04513522 **Trial Status:** COMPLETED **Sponsor Name:** Bristol Myers Squibb **Investigational Product:** Biological: Nivolumab and Biological: Ipilimumab (Yervoy) **Phase of Development:** Phase 4 **Medical Monitor:** Bristol Myers Squibb Medical Monitor / Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the safety and efficacy of nivolumab combined with ipilimumab in intermediate and poor-risk participants with previously untreated advanced renal cell carcinoma (RCC) or metastatic RCC (mRCC) in India. **Secondary Objectives:** To evaluate the time to onset and time to resolution of high grade Immune-Mediated Adverse Events (IMAEs), and to track the number of participants who received immune-modulating medication.

2.2 Background & Rationale To address the clinical need for effective, durable front-line treatments in intermediate and poor-risk patients with advanced or metastatic RCC. This study evaluates the combination of anti-PD-1 (nivolumab) and anti-CTLA-4 (ipilimumab) checkpoint inhibitors specifically within the Indian population to generate robust regional real-world efficacy and safety data, ensuring appropriate risk-benefit profiling for these demographics.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Single Group Assignment) **Blinding:** Open-label (None) **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: 101 participants **Number of Sites:** 11 locations across India **Estimated Study Start:** 17-Dec-2020 **Estimated Primary Completion:** 04-Jan-2024

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histological confirmation of renal cell carcinoma (RCC) with clear cell component including participants who may have sarcomatoid features. 2. Qualifies as intermediate or poor risk by meeting at least one of the prognostic factors as per the International Metastatic RCC Database Consortium (IMDC) criteria. 3. Indian participants with Indian ethnicity living in India. 4. No prior systemic therapy for RCC. 5. Measurable disease lesion by computed tomography (CT) or magnetic resonance imaging (MRI) per Response Evaluation Criteria in Solid Tumors (RECIST) 1.1 criteria.

4.2 Exclusion Criteria 1. Participants with active, untreated, symptomatic central nervous system (CNS) metastases. 2. Major surgery less than 28 days prior to the first dose of study treatment. 3. Participants with an autoimmune disease, or any other condition, requiring systemic treatment with either corticosteroids or other immunosuppressive medications.

(Other protocol-defined inclusion/exclusion criteria apply)

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Nivolumab combined with ipilimumab administered intravenously (typically nivolumab 3 mg/kg + ipilimumab 1 mg/kg every 3 weeks for 4 doses, followed by nivolumab maintenance therapy). **Route of Administration:** Intravenous (IV) Infusion **Duration of Treatment:** Until disease progression, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Routine palliative and supportive care medications, specific treatments for IMAEs (e.g., systemic corticosteroids when indicated for toxicity management). **Prohibited:** Concurrent systemic immunosuppressive therapies or corticosteroids for non-AE management; any concurrent investigational antineoplastic therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, IMDC Risk Assessment, Baseline CT/MRI
Baseline	Day 0	Enrollment, First Infusion, Baseline Safety Labs	Interim Every 3 Weeks Combination Dosing (4 cycles), Safety Labs, IMAE Assessment End of Study Varies End of treatment visit due to progression/toxicity, Survival follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Total Number of Participants Experiencing High Grade Immune-Mediated Adverse Events (IMAE) (Grade 3 to 4 and Grade 5). **Measurement Method:** Clinical assessment tracking IMAEs (diarrhea/colitis, hepatitis, pneumonitis, nephritis and renal dysfunction, rash, and endocrine disorders). Includes events regardless of causality which occurred during the study and follow-up. Only participants receiving immune-modulating medication for treatment of the event are reported, with the exception of endocrine events (included regardless of treatment). Graded via NCI CTCAE where Grade 3 is severe, Grade 4 requires hospitalization, and Grade 5 is fatal.

7.2 Secondary Endpoints * Time to Onset of High Grade Immune-Mediated Adverse Event (IMAE) * Time to Resolution of High Grade Immune-Mediated Adverse Event (IMAE) * The Number of Participants Who Received Immune-Modulating Medication

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring at each dosing visit. Specific monitoring guidelines are utilized for immune-mediated adverse events (IMAEs) such as pneumonitis, colitis, hepatitis, and endocrinopathies. **Serious Adverse Events (SAEs):** Routine 24-hour reporting mechanisms for high grade (grade 3-5) events and events requiring hospitalization. **Data Monitoring Committee (DMC):** Internal safety review committee established by Bristol Myers Squibb.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: All enrolled participants who receive at least one dose of study medication are included in the Safety and Efficacy (ITT equivalent) populations. **Sample Size Justification:** $N=101$, adequately powered to provide meaningful descriptive safety and efficacy statistics within the Indian sub-population. **Handling of Missing Data:** Time-to-event endpoints analyzed using Kaplan-Meier estimates. Patients who initiate subsequent anti-cancer therapy prior to progression are censored at the date of their last evaluable tumor assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Standard risk-based onsite and remote clinical monitoring visits to ensure data integrity and patient safety.

Audit Trail: Continuous eCRF locking and data clarification query resolutions managed by BMS Clinical Data Management.

11. Study Identifiers & Governance

Primary ID: CA209-7C9 **Registry Number:** NCT04513522 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** A Study to Evaluate the Safety and Efficacy of Nivolumab With Ipilimumab in Participants With Untreated Advanced Kidney Cancer Conducted in India **Official Regulatory Title:** A Phase 4 Study of Nivolumab in Combination With Ipilimumab in Patients With Previously Untreated Advanced Renal Cell Carcinoma and Intermediate-or Poor-risk Factors Conducted in India

12. Clinical Framework (Advanced RCC Specific)

Primary Condition: Kidney Neoplasms (Advanced or Metastatic Renal Cell Carcinoma) **Diagnosis Threshold:** Histological confirmation of clear cell RCC component; IMDC intermediate or poor risk **Medical Methodology:** Immune Checkpoint Inhibition (anti-PD-1 + anti-CTLA-4) **Optimization Target:** Disease Control and IMAE Assessment

13. Study Procedures & Methodology

Management Pathway: Standard clinical pathway for advanced mRCC **Education Protocol (HCP):** IMAE management and protocol safety guidelines training **Education Protocol (Patient):** IMAE symptom recognition and urgent reporting procedures **Quality Audit Mechanism:** Routine site monitoring and RECIST 1.1 local/independent radiologic review

14. Observation & Follow-Up Schedule

Total Duration: Until disease progression or unacceptable toxicity (inclusive of survival follow-up) **Onsite Visit Cadence:** Every 3 weeks (Q3W) during combination phase, followed by Q2W or Q4W maintenance **Remote Monitoring Frequency:** Regular safety follow-ups post-treatment termination **Checklist Review Interval:** Imaging (CT/MRI) tumor assessments every 6-9 weeks

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				
Statistician	[Name]	_____	[DD-MMM-YYYY]				